

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Calcipotriol and Betamethasone for the treatment of Psoriasis

Guideline Summary

Summary of NICE and NICE CKS Guidelines for Psoriasis

Assessment and Diagnosis

- Psoriasis is a chronic, immune-mediated inflammatory skin condition, often with a relapsing-remitting course.
- Diagnosis is primarily clinical, based on the appearance of typical skin lesions and distribution.
- Assess:
 - Disease severity (using PASI, DLQI, or BSA involvement).
 - Psychological and social impact.
 - Presence of psoriatic arthritis (ask about joint pain/stiffness).
 - Triggers and comorbidities (e.g., metabolic syndrome, depression).

General Management Principles

- Tailor treatment to individual needs considering:
 - Disease severity and site.
 - Patient preference.
 - Impact on physical, psychological, and social well-being.
- Encourage adherence to treatment and review regularly.

Topical Treatments (First-Line for Mild to Moderate Psoriasis)

- First-line: Fixed combination calcipotriol (vitamin D analogue) and betamethasone dipropionate (a potent corticosteroid).
 - Apply once daily for up to 4 weeks on the body and 1 week on the scalp or face (if licensed).
 - Suitable for adults and children (check age indications).
- Alternatives (depending on area affected or response):
 - Vitamin D analogues alone.
 - Potent corticosteroids.
 - Coal tar preparations.
 - Dithranol (less commonly used).

- Emollients should be used alongside all treatments to maintain skin hydration.

Scalp Psoriasis

- First-line: Medicated shampoos, coal tar, or topical corticosteroids.
- Calcipotriol/betamethasone foam or gel can be considered if licensed and appropriate.

Referral and Specialist Involvement

Refer to dermatology if:

- Diagnosis is uncertain.
- Psoriasis is severe, extensive, or not responding to topical treatments.
- There is significant impact on quality of life.
- Systemic therapy or phototherapy is required.
- Psoriatic arthritis is suspected.

Monitoring and Safety

- Educate patients about potential side effects of topical corticosteroids.
- Monitor for long-term complications with prolonged use (e.g., skin thinning).
- Review treatment response at 4 weeks and modify as needed.

Patient Group Direction

for the supply/administration of

Calcipotriol and betamethasone

for the treatment of

Psoriasis

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

Guideline Summary

PGD Indication	Calcipotriol and betamethasone combination is indicated for the topical treatment of mild to moderate plaque psoriasis in adults. It is used to reduce inflammation, scaling, and erythema, and to improve the overall appearance of psoriatic plaques. This PGD supports its use in patients
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	where systemic therapy is not required, and where a fixed-dose combination topical treatment is appropriate.
Inclusion criteria	- Adults aged 18 years or older. - Confirmed diagnosis of mild to moderate plaque psoriasis. - Suitable for topical treatment with calcipotriol/betamethasone as per clinical assessment. - Patient has provided informed consent. - First presentation or repeat supply where previous response was appropriate and no new contraindications or cautions apply.
Exclusion criteria	- Under 18 years of age. - Extensive psoriasis requiring systemic treatment. - Hypersensitivity to calcipotriol, betamethasone, corticosteroids, or any component of the preparation. - Presence of other skin conditions (e.g., viral, fungal or bacterial skin infections, rosacea, acne vulgaris). - Current use of phototherapy or systemic immunosuppressive agents. - Pregnant or breastfeeding individuals. - Facial, genital or flexural psoriasis.
Cautions	- Use with care in patients with diabetes, glaucoma, or cataracts. - Long-term use may lead to skin atrophy, telangiectasia, and systemic corticosteroid effects. - Avoid occlusive dressings unless directed. - Risk of hypercalcaemia if used excessively. - Avoid concurrent use of other vitamin D analogues. - Advise patients to limit or avoid excessive exposure to either natural or artificial sunlight
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Calcipotriol and betamethasone cream, foam, gel and ointment, 50micrograms/g + 0.5mg/g
Legal category	POM
Storage	Store below 25°C. Do not refrigerate. Keep container tightly closed.
Route/method of administration	Topical application to affected areas of skin only. Ensure formulation chosen is licensed for area to be treated. Patients or carers should be advised to allow calcipotriol with betamethasone preparations to remain on the scalp or skin overnight or during the day, and not to shower or bathe immediately after application.

	Hands should be washed thoroughly after application to avoid inadvertent transfer to other body areas.
Dose and frequency	Apply once daily to affected areas. When using calcipotriol-containing preparations the maximum total dose of calcipotriol is 5 mg (equivalent to 100 g total amount of any formulation) in any 1 week, applied to a maximum of 30% of body surface.
Quantity to be administered and/or supplied	Up to 100g per week, as appropriate to treatment area and patient need.
Maximum or minimum treatment period	Up to 4 weeks continuously. If long-term treatment is required, reassess periodically.
Adverse effects	Burning sensation, irritation, pruritus, folliculitis, skin atrophy, telangiectasia, systemic corticosteroid effects, hypercalcaemia (rare). Refer to SPC and PIL.

Yellow card reporting	Report via https://yellowcard.mhra.gov.uk . Document and inform GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Advice on the nature of psoriasis and explain that treatment is aimed at control of symptoms rather than cure, and that complete clearance of skin lesions may not be possible. It is important that people using topical preparations and their carers understand the fire risk associated with the build-up of residue on clothing and bedding. People using these products should be instructed not to smoke or go near naked flames because clothing or fabric such as bedding or bandages that have been in contact with these preparations or treated skin can rapidly ignite. Washing clothing or fabric at a high temperature may reduce emollient build-up but not totally remove it. Advice to seek medical advice if there is unexplained joint pain or swelling, as this may be a sign of psoriatic arthritis that requires specialist referral. Provide advice on sources of information and support, such as: -The Psoriasis and Psoriatic Arthritis Alliance (www.papaa.org), a national charity which provides information and advice on all aspects of psoriasis -The Psoriasis Association (www.psoriasis-association.org.uk), a national charity and membership organization for people affected by psoriasis.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
1/11/25		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025

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Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Calcipotriol is indicated for the topical treatment of mild to moderate plaque psoriasis in adults. It is used to reduce inflammation, scaling, and erythema, and to improve the overall appearance of psoriatic plaques. This PGD supports its use in patients where systemic therapy is not required, and where a fixed-dose combination topical treatment is appropriate.
Inclusion criteria	- Adults aged 18 years or older. - Confirmed diagnosis of mild to moderate plaque psoriasis. - Suitable for topical treatment with calcipotriol as per clinical assessment. - Patient has provided informed consent. - First presentation or repeat supply where previous response was appropriate and no new contraindications or cautions apply.
Exclusion criteria	- Under 18 years of age. - Extensive psoriasis requiring systemic treatment. - Hypersensitivity to calcipotriol - Presence of other skin conditions (e.g., viral, fungal or bacterial skin infections, rosacea, acne vulgaris). - Current use of phototherapy or systemic immunosuppressive agents. - Pregnant or breastfeeding individuals. - Facial, genital or flexural psoriasis.
Cautions	- Avoid occlusive dressings unless directed. - Risk of hypercalcaemia if used excessively. - Avoid concurrent use of other vitamin D analogues. - Calcipotriol should be used with caution in skin folds as this may increase the risk of side effects
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Calcipotriol ointment and cutaneous solution, 50micrograms/g or ml
Legal category	POM
Storage	Store below 25°C. Do not refrigerate. Keep container tightly closed. Keep the bottle in the outer carton in order to protect from light. Keep the cutaneous solution away from fire or flames (the alcohol base is inflammable).
Route/method of administration	Topical application to affected areas of skin only.

	Calcipotriol Ointment 50micrograms/g should be applied to the affected skin on limbs or trunk once or twice daily. At the beginning of treatment, twice daily (morning and evening) application is recommended. The frequency of application may be decreased to once daily, depending on the response. Ointment has to be applied as a thin layer to affected skin with gentle rubbing to cover the affected area until most of the ointment disappears. The maximum amount of ointment applied should not exceed 100 grams per week. Calcipotriol 50 micrograms/ml Scalp Solution should be applied to the affected areas twice daily (morning and evening). The maximum weekly dose should not exceed 60 ml. If both are used together the total weekly dose of calcipotriol should not exceed 5 mg (for example 40 ml scalp solution plus 60 g of ointment) due to the risk of hypercalcaemia.
Dose and frequency	Apply once or twice daily to affected areas. At the beginning of treatment, twice daily (morning and evening) application is recommended. The frequency of application may be decreased to once daily, depending on the response. Maximum weekly dose: 100g of ointment or 60g of solution.
Quantity to be administered and/or supplied	Up to 100g of ointment or 60ml of solution per week, as appropriate to treatment area and patient need.
Maximum or minimum treatment period	Up to 4 weeks continuously. If long-term treatment is required, reassess periodically.
Adverse effects	Burning sensation, irritation, pruritus, folliculitis, skin atrophy, telangiectasia, systemic corticosteroid effects, hypercalcaemia (rare). Refer to SPC and PIL.

Yellow card reporting	Report via https://yellowcard.mhra.gov.uk . Document and inform GP as appropriate.
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